

Master Context Document

RxContent — Pharmacist TikTok Script Generator

Purpose: This document is the single source of truth for the RxContent project. It should be read at the start of every new session to restore full context. It captures who the user is, what has been built, why key decisions were made, and what is planned next.

Last Updated: May 19, 2026

How to Use This Document

Starting a new session (any AI): Share this document. It restores full context on the creator, the tool, all decisions made, all rules, and what is planned next.

Scripting tasks: Share this document AND `HOOK_FRAMEWORKS.md`. The master context covers who, what, and why. The hooks framework covers the structural rules needed to actually write a script. Both are required for scripting work.

Iterating on a specific existing script: Also share the relevant script file from `/scripts/`.

Creator analysis or Video Lab work: Also share the relevant analysis file from `/analysis/`.

This document does NOT need to be regenerated — it is updated in place at the end of each session. Always use the most recent version.

1. About the Creator

Background: Mixed-race (Black/White) pharmacist, born and raised in Brooklyn, NY by a single mother in a lower-class household. Licensed pharmacist since 2013, currently working at CVS. Prior experience includes one year at a hospital pharmacy and four years as a pharmacy intern in a hospital during pharmacy school. This clinical background — particularly the hospital internship — gives a depth of pharmacological knowledge that retail-only pharmacists do not have.

Why this matters for content: The combination of street-level relatability (Brooklyn upbringing, working-class background) and clinical authority (PharmD, hospital experience, 13+ years dispensing) is a rare and powerful positioning. Most healthcare creators are either relatable but not credentialed, or credentialed but not relatable. This creator is both.

Goals — short term: Build TikTok Shop affiliate income as a side hustle that grows into a primary or co-primary income source. The goal is for TikTok to eventually replace the CVS job or reduce it to part-time/per diem.

Goals — long term: Become a trusted healthcare resource for people who do not have easy access to healthcare providers. Educate people about supplements and health products that can genuinely improve quality of life. Be living proof of what is possible for people from Brooklyn and similar backgrounds — not just as a pharmacist, but as a creator and entrepreneur. Many people from the creator's community already look up to them but may assume success required innate advantages. The TikTok journey is meant to show that a repeatable, learnable system can produce real results for anyone willing to put in the work.

Content philosophy: The healthcare content (Rx Content / TOF) is the primary lane — it is where the credential creates the most differentiation and the highest trust. The BOF deal content (Shop Script) is a secondary lane, studied and built because it is accessible to anyone and because understanding it makes the creator a more complete TikTok Shop operator. The long-term vision includes teaching others what has been learned.

On-screen positioning: Male pharmacist. The male credential is a genuine competitive advantage in categories like hair loss, testosterone, men's skincare, ear care, and men's sexual health — categories where female creators cannot occupy the same authority position. The introductory phrases tested are:

- *“I’m a pharmacist. I talk to doctors and patients every day, and I’m going to tell you exactly what actually works.”* (Clinical Insider)
 - *“I’m a pharmacist. I counsel patients on this stuff every single day. Here’s what I actually tell them.”* (Authority + Relatability)
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2. The Tool — What Was Built and Why

Tool name: RxContent (Pharmacist TikTok Script Generator)

URL: pharmascript-tonqparb.manus.space

Stack: React 19 + Tailwind 4 + Express 4 + tRPC 11 + MySQL (Drizzle ORM) + Manus Auth

Design language: Clinical Command Center — deep navy (#0A1628) background, electric teal (#00D4AA) accents, IBM Plex Mono for script output, Space Grotesk for headings. Chosen because it feels like a professional internal tool, not a consumer app.

The tool was built because manual script writing is slow and inconsistent. The goal was to encode the patterns from top-performing healthcare creators into a system that generates scripts at the quality level of those creators, but adapted to this creator’s specific voice and credential. Every design decision in the tool was made to serve that goal.

Four Modules

Module	Purpose
Rx Content	Generate original pharmacist scripts using 14 hook frameworks derived from real viral healthcare videos
Shop Script	Generate BOF (Bottom-of-Funnel) deal scripts using verbatim line banks from top-performing deal creators
Vet Product	Analyze a TikTok Shop product listing for clinical credibility, red flags, and affiliate viability
Video Lab	Transcribe and analyze any TikTok video; Clone Mode rewrites it in the creator’s voice; Iterate Mode generates 70/20/10 variations

3. Rx Content — Healthcare Script Generator

The Core Architecture

Every script generation runs two steps in sequence:

1. **researchProduct()** — calls the LLM to extract mechanism of action, clinical backing, dosing facts, citations, and the key “gap” (the common mistake most people make). The prompt demands specific numbers, named studies, and PMID/DOI references. It explicitly rejects vague answers.
2. **generateSingle()** — uses the research output as grounding context. The LLM writes the full script from the research facts, not from scratch. The hook-first constraint is enforced in the prompt: the verbal hook must be the first spoken words, no preamble.

Fact-checking stance: The LLM draws on training data, not live PubMed queries. For well-studied ingredients (creatine, magnesium glycinate, vitamin D, omega-3, NAD+), the claims are reliable and consistent with published science. For newer compounds (urolithin A, spermidine, NMN), the creator applies their own pharmacist judgment. The creator always opens PubMed links before citing them — a fabricated PMID is self-correcting because the link will not resolve. No automated fact-checking layer is needed; the creator’s clinical training is the final filter.

The 14 Hook Frameworks

The frameworks are organized in two tiers. Tier 1 hooks are high-volume and proven. Tier 2 hooks are secondary or situational.

Tier 1 — Heavy Hitters:

Hook	Reference Creator	Key Pattern
Instruction / Correction	@adoseofwellness (Riva) — 12.8M views	“Here’s how to take X the right way” / “Most people are taking X wrong”
Symptom Checklist	@adoseofwellness (Riva) — 3.9M views	Visual symptom list → cause → product
Suppressed Knowledge	@adoseofwellness (Riva)	“What your doctor doesn’t tell you about X”
After 1 Month Formula	@drew.review — 7.9M views	”[Product] after 1 month — from a pharmacist”
Trend or Trash	@drew.review	“Is X worth it? A pharmacist looks at the evidence”
Comparison Hook	@rphreviews	“X vs. Y — a pharmacist breaks it down”
Warning Signs Hook	@rphreviews	“Stop doing X if you have Y”
How Do You Know	@rphreviews	“How do you know if you have [condition]?”

Tier 2 — Secondary / Situational:

Hook	Status	Notes
Age Reversal	No confirmed reference video	Framework inferred — needs Kalodata validation
Dosing Authority	No confirmed reference video	Framework inferred
Forbidden Knowledge	No confirmed reference video	Framework inferred
The Mechanism Reveal	Partial reference	
Before / After Protocol	Partial reference	
The Comparison Showdown	@rphreviews	Confirmed — Ordinary vs. Medicube Volufiline

Key Structural Rules (Universal)

Every script must contain a **Gap** — a statement of what most people are missing, doing wrong, or not aware of. The product fills this gap. Without the gap, the recommendation feels like an ad rather than expert advice.

Product introduction timing is late by default. For education-heavy hooks, the product should not appear until the final third of the script. For symptom/pain-point hooks, the product can appear earlier once the cause is identified.

Hook-first constraint: The verbal hook must be the first spoken words. No preamble before the hook. This is enforced in the generation prompt.

Problem section rule: The problem section must name a specific mechanism failure — not a behavioral observation. BAD: “Most people expect instant results.” GOOD: “Most people take it without fat so they absorb less than 20%.” This was a specific prompt improvement made after observing generic filler lines in early outputs.

TOF curiosity loop rule: In top-of-funnel scripts, the product name, brand, and visual are NEVER revealed until the final section. Structure: hook the viewer → open curiosity loop (what is causing this?) → close with education (here is why) → open second loop (what actually helps?) → close with solution mechanism → THEN introduce the product as the delivery vehicle. Revealing the product early kills watch time.

Exception: comparison hook — product is shown upfront, but the verdict is withheld until the end.

Triple Hook Strategy: Every script deploys three simultaneous attention triggers in the first 2 seconds: (1) **Visual hook** — what is physically on screen; (2) **Spoken/verbal hook** — the first words out of your mouth; (3) **Text overlay hook** — on-screen text that approaches the same topic from a different angle, never repeating the spoken words verbatim. The tool generates 3 text overlay options per script: question format, provocative statement, and stakes framing. Always generate all three before filming and pick the one that matches your delivery energy.

Every script deliverable includes a caption and 5 hashtags. Caption: max 2 lines, subtle CTA acceptable (“definitely worth a try”) but never transactional (“tap the orange cart”). Hashtags: exactly 5 — brand name, ingredient/product name, 2 pain point tags, 1 general health/pharmacy tag.

New product scripts include 3–4 verified PubMed studies. Always pull the actual PubMed or PMC page before citing — never cite from memory or training data alone. Distinguish between mechanism studies (in vitro/animal) and human RCTs when describing evidence strength. Studies map to specific claims in the script and are used for on-screen study pop-ups.

TOF product intro framing: Do not say “if you’re looking for a great recommendation.” TOF viewers have not done prior research and do not know they need the product yet. Instead use: “This is the one I use daily. This is the one I recommend to my patients.” The credential does the selling — not the pitch.

Pain points before the reveal: In symptom-reframe hooks, list ALL symptoms first, then name the cause. Do not name the cause mid-hook. Let the viewer accumulate recognition before the reveal lands.

Let visuals carry the load: If a diagram or on-screen graphic shows something, do not also explain it verbally. Name one or two examples verbally, then let the visual do the rest. Verbal and visual should complement each other, not duplicate.

CTA rule: Once the CTA starts, never go back to product features. Features must come before the CTA. CTA structure: social proof + urgency + link. Keep it short.

The Misdirection Technique

An optional toggle in the UI. When enabled, adds a line that corrects an overclaim to build credibility. The rule is strict:

- **BAD (hedging):** “Everyone says this works but honestly it works for many people.”
- **GOOD (credibility-building correction):** “Everyone says you need to do a loading phase with creatine — but honestly a consistent 3-5g daily dose reaches the same saturation in 28 days with none of the GI side effects.”

The misdirection must correct a real overclaim in the category. It should make the creator sound more trustworthy, not less confident.

Why No Verbatim Line Bank for Rx Content

The BOF generator uses verbatim lines from real creator videos because the deal script structure is formulaic and the exact language matters. The Rx Content generator does NOT use verbatim lines because:

1. Healthcare scripts need to sound like this specific creator — not like a line pulled from another pharmacist’s video.
2. The LLM writing the hook from scratch using the research context and voice guidelines produces better results than assembling from examples.
3. Verbatim lines from other creators would undermine the authenticity that is the core value proposition.

Study Verification Standard

All studies must be verified by navigating to the actual PubMed or PMC URL before citing. Never recommend a study from memory or training data alone — a fabricated PMID will not resolve and will undermine credibility. Confirmed studies on file:

Product	Study	PMID/PMC	Claim
Neuro Gum	Kamimori 2002	PMID 11839447	Buccal absorption of caffeine
Neuro Gum	Giesbrecht 2010	PMID 21040626	L-theanine + caffeine synergy
Medicube Kojic Acid	Chang 2009	PMC2705500	Tyrosinase inhibition mechanism
Medicube Kojic Acid	Sarkar 2013	PMC3663177	Fades dark spots (human evidence)
Medicube Kojic Acid	Lajis 2012	PMC3468271	Depigmenting effect
Magnesium/Cortisol	Sartori 2012	PMID 21835188	Magnesium reduces stress response
Magnesium/Cortisol	Pickering 2020	PMID 33260549	Magnesium deficiency and stress
Magnesium/Cortisol	Boyle 2017	PMID 28445426	Magnesium and anxiety (meta- analysis)
Magnesium/Cortisol	Abbasi 2012	PMID 23853635	Magnesium and sleep quality

4. Shop Script — BOF Deal Generator

The BOF generator uses a verbatim line bank built from deep analysis of top-performing deal creators. The line bank is in `client/src/lib/bofLineBank.ts`. Each line is tagged with creator, frequency rating, and slot position.

Tracked BOF creators and their primary patterns:

Creator	Primary Hook	Signature Element
@momfindsbyfaith (Faith)	Reverse Psychology (“Do Not Get This”)	Coupon gamification — “some of you even have coupons today”
@dealscope	Comparison / Upgrade (“Do Not Buy Small One”)	TikTok Glitch hook
@blackfridaybrian (Brian)	Fake Outrage	Returning This variant with outrage framing
@welearn2earn	Deal Alert / Triple Discount	Quantity math
@dealssforeveryone	Bundle / Motherload	Flash sale urgency

Faith’s coupon gamification (92% of her videos):

“Some of you even have coupons today. Not everyone sees those coupons and it’s for a limited time so you’ll have to go to checkout to see if you have one.”

Key insight: Faith never confirms a coupon exists — she uses conditional language (“some of you”, “not everyone sees”) to drive checkout clicks without making a false promise.

The “Before I Reach for the Pill Bottle” Framework: One of the most powerful content frames for a pharmacist creator. A pharmacist recommending something *other* than a drug is a pattern interrupt. The credential becomes the reason to trust the non-drug recommendation. Template: *“As a pharmacist, the first thing patients ask me for [condition] is medication — but before I reach for the pill bottle, here’s what I actually tell them to try first.”*

5. Clone Mode — Key Decisions

Clone Mode transcribes a viral TikTok video and rewrites it in the creator’s pharmacist voice. Critical decisions made:

Hook switching was removed. The original implementation generated 5 rewrites using different hook frameworks. This was wrong — Clone Mode’s purpose is to preserve what made the original work, not to explore alternatives. Hook cycling was

removed. Clone Mode now generates exactly one rewrite that preserves the original video's hook framework and section structure.

Why: When you find a viral video, the proven structure is the asset. Switching hooks defeats the purpose. If you want to explore alternative hooks, that is what Iterate Mode is for.

The correct Clone Mode workflow: Clone first (preserve structure, adapt voice) → Iterate second (explore variations) → Generate when you want to own the angle completely from scratch.

6. Video Lab — Creator Analysis

The Video Lab transcribes and classifies TikTok videos. It is used for:

- Analyzing viral videos before cloning or iterating
- Studying competitor/reference creator scripts
- Understanding what hook framework a video uses

Methodology rules (non-negotiable):

1. Captions are not hooks. The verbal hook is what the creator says in the first 1-2 seconds on camera. Captions are SEO copy and routinely do not match the actual opening line.
2. Only analyze videos from the creator's own profile grid. TikTok's sidebar shows other creators' videos — these contaminate the analysis.
3. A creator onboarding is not complete until the generation pipeline is updated. Documentation alone does not count.

Required Workflow for Any TikTok Video Analysis

The only way to accurately extract a spoken hook, pacing, section order, tonality, CTA structure, and on-screen text sequence is to download and transcribe the actual video audio. TikTok page metadata (captions, titles, hashtags) is discovery copy — it does not reflect what the creator says on camera or how the video is structured.

Step 1 — Download the video using yt-dlp:

```
yt-dlp "https://www.tiktok.com/@handle/video/1234567890" -o  
"/home/ubuntu/upload/video_name.mp4" --no-playlist
```

Step 2 — Transcribe and analyze:

```
manus-analyze-video "/home/ubuntu/upload/video_name.mp4" "Transcribe this  
TikTok video word for word exactly as the creator speaks. Also note every  
on-screen text overlay, graphic, diagram, product shot, and study popup that  
appears. Note the order of everything."
```

This process works for any public TikTok video — no login required. yt-dlp downloads directly from the public URL.

What full transcription captures that captions cannot:

- The exact spoken hook (word for word, first 1–3 seconds)
- The section order (hook → authority → education → gap → product reveal → CTA)
- The exact moment the product is named or shown on screen
- Every on-screen text overlay and when it appears
- Every graphic, diagram, or study popup and what it says
- The tonality — conversational vs. authoritative vs. urgent
- The exact CTA language and its length
- Whether the product is revealed early (comparison hook) or late (TOF curiosity loop)

For coaching video analysis (e.g., Coach Ruben breakdowns): Same process. Download the coaching video and run `manus-analyze-video` with a prompt asking for both the coach's feedback AND the original creator video content being reviewed. This captures the exact changes recommended and the specific lines being critiqued.

Never analyze from captions alone. If a video cannot be downloaded, note it as unverified and do not extract hook lines from it.

7. Tracked Healthcare Creators and Analyzed Videos

Note on reference video analysis: When a script is built from reference videos, the source video links are included in the script doc under a “Reference Videos Analyzed” table. The following videos have been downloaded and fully transcribed (not just captioned) for analysis:

Creator	Product/Topic	Used For
@rphreviews	Magnesium/Cortisol (AshwaMag)	Magnesium v3 stress dysregulation section
@adoseofwellness (Riva)	Magnesium/Cortisol	Magnesium v3 stress dysregulation section
@rphreviews	Medicube vs. Dr. Melaxin	Comparison script Version C (exact remake)
@momfindsbyfaith (Faith)	Medicube vs. Dr. Melaxin	Comparison script Version B (exact remake)
@momfindsbyfaith (Faith)	Loaded Tea vs. Bloom	Analyzed and assessed — missed the mark (see loaded-tea-vs-bloom-comparison.md)

7b. Tracked Healthcare Creators

These creators were analyzed to build the Rx Content hook frameworks:

@adoseofwellness (Riva) — Female pharmacist. Primary source for Instruction/Correction, Symptom Checklist, and Suppressed Knowledge hooks. Reference video: 12.8M views on creatine instruction/correction. Key pattern: first 70% of video is pure education; product appears only after trust is established. Has done energy drink content (Loaded Tea vs. Bloom comparison — assessed as missing the mark: only linked one product, spent too long on sucralose/prebiotic contradiction, no mechanism education, no on-screen study popups).

@drew.review (Drew) — Male pharmacist/healthcare creator. Primary source for After 1 Month Formula and Trend or Trash hooks. Reference video: 7.9M views. Key pattern:

personal testing narrative combined with clinical authority.

@rphreviews — Male pharmacist (17+ years). Primary source for Comparison Hook, Warning Signs, How Do You Know, and Comparison Showdown. Every video contains affiliate disclosure. Key pattern: comment-reply format builds parasocial trust; ingredient-first framing. The “how-do-you-know” hook was identified and added to the framework after analyzing this creator. Has NOT done energy drink content — energy drink space is an open lane for a credentialed pharmacist.

@faithfuldoc (Faith’s Rx content) — Healthcare creator. Analyzed for Rx-style content patterns.

8. Key Product Categories and Analyzed Products

Products Fully Analyzed (Verified Ingredient Profiles on File)

Product	Key Story	Comparison Potential
Neuro Gum	Buccal caffeine absorption, L-theanine synergy	vs. regular caffeine/coffee
Medicube PDRN Kojic Acid Serum	Tyrosinase inhibition, kojic acid timing (28-day cell turnover)	Instruction/correction angle
Medicube PDRN Pink Collagen Multibalm Stick	Volufiline (Anemarrhena asphodeloides root), PDRN, liposomal delivery	vs. Dr. Melaxin
Dr. Melaxin Cemenrete Volume Multi-Balm	Rebornic (calcium + Vitamin D, activates Vitamin D receptors for cell turnover + elasticity), hydrolyzed collagen, adenosine, elastin	vs. Medicube
The Loaded Tea Shop	Green tea caffeine (natural, contains L-theanine), full B-vitamin stack (400% B12, 290% B6, 190% B3), D3 + MK-7 K2, FDA-registered/cGMP facility	vs. Celsius, vs. Bloom
Bloom Sparkling Energy	Natural caffeine (green coffee bean + green tea), explicitly dosed L-theanine, galactomannan prebiotic, ginseng, lychee extract. Wins on functional extras; Loaded Tea wins on B-vitamin dosing and transparency	vs. Loaded Tea
Celsius	MetaPlus blend — 6 university studies (thermogenic effect real but caffeine-driven), proprietary blend hides exact doses, caffeine anhydrous (synthetic). Loses on transparency vs. Loaded Tea	vs. Loaded Tea

Key product insight — Herbalife association: The loaded tea category originated from Herbalife nutrition clubs. The Loaded Tea Shop is not affiliated with Herbalife.

This fact is NOT mentioned in any scripts — it wastes video time and introduces doubt for viewers who know about it. If it comes up in comments, handle it with a comment reply video.

Key product insight — Firming vs. volumizing: Rebornic (Melaxin) improves cell turnover and elasticity — it does not replace lost volume. Volufiline (Medicube) physically increases subcutaneous fat cell volume. These are fundamentally different mechanisms. This distinction is the pharmacist-level insight that makes the Medicube vs. Melaxin comparison feel like expert advice rather than a sales pitch.

Key product insight — Caffeine anhydrous vs. green tea caffeine: Caffeine anhydrous is synthetic isolated caffeine with no natural modulation — it spikes fast and drops hard. Green tea caffeine naturally contains L-theanine alongside it, which smooths the stimulant effect. This is the core clinical story for all Loaded Tea scripts.

Pronunciation note: Volufiline = “vol-yoo-FY-leen” (4 syllables, emphasis on third). Say it matter-of-factly — the confidence is part of the authority signal.

8b. Original Product Categories

The creator’s highest-priority product categories, ranked by competitive advantage (original ranking — see Section 8 for fully analyzed products):

1. **Hair loss** — Highest priority. Men 25-45 are highest-intent buyers. Almost no credentialed male creators. Minoxidil, DHT-blocking supplements, derma roller, ketoconazole shampoo.
2. **Men’s skincare** — Underserved. Male authority drives higher purchase intent from male viewers.
3. **Supplements (energy/longevity)** — NAD+, creatine, magnesium glycinate, collagen, omega-3. Well-studied, strong clinical backing, high repeat purchase.
4. **Ear care** — Viral category on TikTok. Clinical authority is extremely high. Otoscope camera is currently viral with no credentialed male creator.
5. **Oral health** — One of the most viral pharmacy categories. Almost no pharmacists in it.
6. **Men’s sexual health / testosterone** — High CPM, low competition from credentialed creators. Male authority is a requirement.

7. **Sleep & recovery** — Mouth tape is currently extremely viral with almost no credentialed creators.
 8. **Pain relief** — Topical diclofenac (Voltaren), compression gear, foam roller.
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9. Decisions Made and Why

Herbalife context excluded from all Loaded Tea scripts — decided May 19, 2026. The association wastes video time and introduces doubt. Every sentence must have a purpose. Comment reply video is the right format if it comes up in comments.

Comparison hook with dual CTA (both products linked) — when two legitimate products serve different use cases, both should be linked in the video. “Which one is right for you” framing is more credible than picking a single winner and captures two buyer types in one video. Established from the Loaded Tea vs. Bloom comparison.

Ingredient-form hook added as inferred framework — no confirmed high-performing reference video exists for this hook. Scripts using it include a delivery note explaining how it should sound. Inferred frameworks are lower confidence than hooks with verified reference videos.

Faith’s Loaded Tea vs. Bloom video assessed as missing the mark — analyzed May 19, 2026. Key failures: only linked one product, spent 40+ seconds on the sucralose/prebiotic contradiction, no mechanism education, no on-screen study popups, rambling in the Loaded Tea section. What was kept: sucralose/prebiotic contradiction (compressed to 2 sentences), showing the drink being made on camera, caffeine context line, customizable dose angle.

Celsius MetaPlus clinical data — honest assessment — 6 university studies show real thermogenic effect, but it is almost certainly caffeine-driven. The proprietary blend hides exact doses. The studies were funded by Celsius. Loaded Tea wins on transparency and B-vitamin dosing. This is the pharmacist conclusion, not a dismissal of Celsius.

No verbatim line bank for Rx Content — decided because healthcare scripts need to sound like this creator specifically. The LLM writing from research context produces better, more authentic output than assembling from another pharmacist’s lines.

Hook-first constraint in generation prompt — added after observing that the LLM would sometimes write preamble before the hook, burying the scroll-stopper. The constraint is enforced in the prompt, not post-generation.

Problem section must name a mechanism failure — added after observing generic filler lines like “most people expect instant results.” The prompt now includes BAD/GOOD examples and a hard rule.

Misdirection must correct an overclaim, not hedge — the original Clone Mode rewrite was softening claims (“works for many people”) instead of correcting overclaims to build credibility. Fixed with an explicit BAD/GOOD example in the prompt and a CONFIDENCE RULE that the rewrite must match the energy of the original.

Clone Mode generates one rewrite, not five — hook cycling was removed because it defeats the purpose of cloning a proven video. One rewrite, original framework preserved.

`assembleScriptFromSections` is a utility, not the generation path — the LLM’s `fullScript` is used directly. The section fields (verbalHook, problem, etc.) are excerpt copies for the breakdown panel and inline editing only. This preserves the full 9-section structure while preventing hook burial.

Post-generation structure check deferred — requires validated real examples for all 14 hooks first. Sequentially dependent on Kalodata deep dive.

9b. Finalized Scripts (Ready to Film)

All finalized scripts live in `/home/ubuntu/pharma-script-gen/scripts/`. Each includes spoken hook, 3 text hook options, visual hook note, caption, hashtags, and verified study citations where applicable.

File	Status	Hook Type
neuro-gum-instruction-correction.md	Filmed ✓	Instruction/Correction
neuro-gum-comparison-hook.md	Ready to film	Comparison Hook
medicube-deodorant-symptom-reframe.md	Ready to film	Symptom Checklist
medicube-kojic-instruction-hook.md	Ready to film	Instruction/Correction
magnesium-cortisol-v2.md	Ready to film	Symptom Checklist (Coach Ruben feedback applied)
magnesium-cortisol-v3-stress-dysregulation.md	Ready to film	Symptom Checklist + Stress Dysregulation bridge (iteration of v2)
medicube-vs-melaxin-comparison.md	Ready to film	Comparison Hook — 3 versions: Best-Of Hybrid (A), Faith exact remake (B), Creator 2 exact remake ©
loaded-tea-vs-bloom-comparison.md	Ready to film	Comparison Hook — Dual-CTA, both products linked, Faith's video analyzed and assessed
loaded-tea-trend-or-trash.md	Ready to film	Trend or Trash — Loaded Tea Shop, green tea caffeine + L-theanine mechanism, full B-vitamin stack
celsius-vs-loaded-tea-comparison.md	Ready to film	Comparison Hook — Celsius vs. Loaded Tea, fair pharmacist breakdown, Loaded Tea wins on transparency and B-vitamin dosing
loaded-tea-ingredient-form.md	Ready to film	Ingredient Form Hook — green tea caffeine vs. synthetic caffeine anhydrous, L-theanine mechanism, full B-vitamin stack. Inferred framework (no reference video — delivery notes included in script)

Original magnesium transcript (pre-coaching) saved as `magnesium-cortisol-original.md` for reference.

Script doc standard: When a script is built from reference videos, the source video links must be included in the script doc under a “Reference Videos Analyzed” table. This was established May 14, 2026.

9c. Script Quality Standards — Non-Negotiable Rules

Established May 16, 2026 after fabricated ingredients were written into a script that was filmed before the error was caught. These rules apply to every script without exception.

1. Ingredient Verification — No Exceptions Before writing any script that mentions specific product ingredients, the full ingredient list must be verified against a primary source: INCIDecoder, the brand’s official product page, or a major retailer listing (Amazon, Ulta, etc.). Never rely on memory, transcription summaries, or what “similar products typically contain.” Every ingredient named in a script must be traceable to a confirmed source. The script doc must include a “Verified Ingredients” section with the source URL.

2. Verbatim Remakes Must Be Verbatim When a script is labeled as an exact remake of a creator’s video, it must be a true word-for-word adaptation. No condensing, no dropping ingredients, no paraphrasing mechanisms. The only permitted changes are: swapping the creator’s credential for the pharmacist credential, and adjusting “I” statements to match the creator’s voice. If a creator names a specific ingredient, mechanism, plant source, or patented complex — it stays in. If it was in their video, it was there for a reason.

3. No Generic Filling Every script must be built from: (a) the specific product’s verified ingredients, (b) content we have actually studied and analyzed together, and © proven frameworks and hooks from this project. Generic skincare claims, placeholder mechanisms, or “typical product” language that is not specific to the actual product being scripted are not acceptable.

4. Full Effort on Every Script Every video matters, especially early on when the channel is building. Each script should reflect the full body of knowledge accumulated in this project — the creator analysis, the hook frameworks, the coaching feedback,

the study verification standard, the TOF rules, the CTA rules. No shortcuts. If a script takes longer to write correctly, that is the right tradeoff.

5. When in Doubt, Verify Before Writing If there is any uncertainty about an ingredient, a mechanism, a study claim, or a creator's exact words — stop and verify before writing it into the script. Guessing and correcting after filming is not an acceptable workflow.

10. Kalodata Roadmap — Planned Upgrades

Everything planned once Kalodata access is obtained. See `/home/ubuntu/kalodata-roadmap.pdf` for the full document.

Step 1 — Healthcare Creator Deep Dive (Re-Analysis)

Re-run full script analysis on Drew, Riva, Faith's Rx content, and rphreviews — filtered by top GMV videos from Kalodata, not by views/saves. Extract: verified section sequences for all 14 hooks, verbatim opening lines (not for line bank — for framework validation), new hook patterns, and confirmed real examples for currently unanchored hooks (Age-Trigger, Forbidden-Knowledge, Dosing-Protocol).

Step 2 — Video Lab Batch Analyzer

Build batch URL input in Video Lab. Paste 50-100 URLs from Kalodata export. Tool transcribes and classifies each against the 14 frameworks. Surfaces: which frameworks appear most in high-converting videos, unclassified patterns (candidates for new hooks). Technical spec: 1-2 second delay between requests (rate limiting), graceful handling of private/deleted videos, skip videos over file size limit.

Step 3 — Hook Framework Performance Scoring

Once enough data points exist: score each framework by average GMV per video in the niche. Surface as a recommendation when selecting a hook for a product.

Step 4 — Product Category Gap Analysis

Cross-reference products tracked creators are converting on against products currently being promoted. Identify untapped opportunities before they saturate.

Step 5 — JSON Schema + Post-Generation Structure Check

Replace generic section structure with exact, hook-specific JSON schemas for each of the 14 frameworks. Post-generation validator flags structural violations (buried hook,

generic problem section, CTA appearing before mechanism). Dependent on Step 1 being complete.

Step 6 — Personal Performance Feedback Loop

Log posted videos against hook framework, product, and GMV outcome. Build personal performance dataset over time. Creator's own conversion data may differ from tracked creators due to specific audience and credential positioning.

Sequencing: Steps 1 and 2 run in parallel. Steps 3 and 4 follow from that data. Step 5 is dependent on Step 1. Step 6 starts immediately once posting begins.

11. What Is NOT Built Yet (Future Roadmap)

- **Script Advisor (floating chat panel)** — discussed but not built. Would be a chat interface with full system context injected. Best for bounded questions: compliance checks, section critique, hook selection. Not a replacement for strategic analysis in a full session.
 - **Kalodata integration** — all of Section 10 above.
 - **Video Lab batch analyzer** — see Step 2 above.
 - **Per-video performance logging** — see Step 6 above.
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12. How to Use This Document

At the start of any new session working on this project, paste or reference this document to restore full context. The agent reading this document should treat it as the authoritative record of decisions made and should not re-debate closed questions without new information.

When something important changes — a new decision is made, a new creator is analyzed, a new framework is added, a roadmap item is completed — this document should be updated to reflect it.

The document lives at `/home/ubuntu/master-context-document.md` and `/home/ubuntu/master-context-document.pdf`.

Update protocol: This document is updated at the end of every session. The filter for what belongs here: decisions made and the reasoning behind them, universal rules that apply to all future scripts, new creators analyzed and key learnings, new product categories, finalized scripts list, roadmap changes, and coaching feedback that produced framework-level rules. Individual script drafts, raw transcripts, video analysis notes, and tool code changes do NOT belong here — they live in their dedicated files.

What does NOT belong here: Full script text (lives in `/scripts/`), raw video transcripts, coaching session notes for a single video, tool bug fixes, code changes. If it is specific to one video or one session, it belongs in a dedicated file — not here.